BHAT FOUNDATION WELFARE SOCIETY

FREE MULTI-SPECIALTY MEDICAL CAMP

PROJECT COMPLETION & DONOR IMPACT REPORT

06–07 July 2026
Ibn Sina Hospital, Ompora, Budgam

Documented headline output: 400 patients received free consultation

BHAT FOUNDATION WELFARE SOCIETY | IBN SINA HOSPITAL

1. Executive Summary

Impact at a Glance
400 patients received free consultation | 2 programme days | 06–07 July 2026 | 1 documented healthcare partnership | Free consultation, check-ups and examinations | Referral guidance for patients requiring further care

Bhat Foundation Welfare Society, in collaboration with Ibn Sina Hospital, conducted a two-day Free Multi-Specialty Medical Camp on 06–07 July 2026 at Ibn Sina Hospital, Ompora, Budgam. The intervention focused on improving access to medical consultation and examinations for members of the general public.

The strongest verified quantitative result in the supplied documentation is 400 patients receiving free consultation. The documented service pathway also included medical check-ups and examinations, professional medical guidance, identification of patients who may require additional treatment or surgical intervention, referral guidance, and preventive-health messaging.

The available evidence demonstrates direct service delivery and an institutional healthcare partnership. The supplied event documentation does not provide a verified demographic breakdown, number of doctors or volunteers, number of referrals issued, referral completion, medicines or diagnostics supplied, expenditure, or quantified indirect reach. These fields are therefore not estimated in this report.

Evidence Integrity Note
The event report identifies the venue as Ibn Sina Hospital, Ompora, Budgam. Photographic evidence supplied with the report contains GPS overlays showing Railway Station Road, Srinagar, on 06 July 2026. Because this differs from the stated venue, the photo metadata should be reconciled against the original event records before external publication.

2. Project Background

The intervention was designed around a documented community-health access need: providing members of the public with free medical consultation, examination and guidance through a healthcare-institution setting. The supplied report frames the activity as a community-oriented public-health intervention intended to reduce immediate barriers to specialist consultation and support early identification of health concerns.

The local geographic context documented for the programme is Ompora, Budgam. The report does not provide a baseline health-needs assessment, prevalence statistics, socioeconomic profile, or beneficiary demographic breakdown. Those data should be added if available rather than inferred.

3. Problem Statement

Problem	Need	Intervention
Access to professional medical consultation may involve direct financial and practical barriers for community members.	Provide an accessible opportunity for medical consultation, examinations and health guidance.	Two-day free medical camp providing consultation, check-ups, examinations, identification of further-care needs and referral guidance.

Evidence status: The problem and intervention logic are documented in the supplied event report. A quantified baseline describing the size of the unmet need is not available.

4. Project Objectives

Primary objectives

- Provide free healthcare consultation to members of the public.
- Improve access to medical examinations and professional medical advice.
- Support identification and early diagnosis of health concerns.

- Identify patients requiring additional treatment or surgical intervention and guide them towards appropriate referral pathways.

Secondary objectives

- Increase awareness regarding preventive healthcare and regular medical check-ups.
- Reduce the immediate consultation-related financial burden for participating beneficiaries.
- Strengthen community-oriented public-health outreach through institutional collaboration.

5. Target Beneficiaries

Category	Number	Description	Direct / Indirect
General public / patients	400 documented service recipients	Patients who received free consultation during the camp	Direct
Indirect beneficiaries	[INFORMATION REQUIRED]	Family/community members benefiting indirectly from information or referral support, if documented	Indirect

The source documentation identifies the target group as the general public. Age, gender, socioeconomic status, disability status and other demographic categories were not supplied.

6. Geographic Coverage

Geographic level	Documented coverage
District	Budgam
Locality / venue	Ibn Sina Hospital, Ompora, Budgam
Additional geographic evidence	Supplied photographs contain GPS overlays identifying Railway Station Road, Srinagar, 191111, dated 06 July 2026; this requires reconciliation with the event venue record.

The report should use the Ompora, Budgam venue as the programme location only after the photographic-location discrepancy is checked against the original attendance, venue, or permission documentation.

7. Implementation Methodology

Stage	Documented implementation
1. Planning	Community-health outreach was organised as a two-day free medical camp in collaboration with Ibn Sina Hospital.
2. Community mobilisation	Community outreach is identified as an input; the specific mobilisation channels and counts are [INFORMATION REQUIRED].
3. Stakeholder coordination	Bhat Foundation Welfare Society collaborated with Ibn Sina Hospital.
4. Team deployment	Medical professionals and healthcare staff are documented as providing services; team size and volunteer count are [INFORMATION REQUIRED].
5. Main intervention	Consultations, check-ups, examinations, health guidance and identification of further-care needs were provided.
6. Follow-up	Patients requiring further treatment or surgery were guided towards appropriate referrals; referral completion is [INFORMATION REQUIRED].
7. Documentation and monitoring	Photographic documentation and the supplied event report provide evidence of implementation; attendance/registration and financial records were not supplied.

8. Activities Conducted

No.	Activity	Date	Location	Participants	Purpose	Output	Evidence
1	Free medical consultations	06–07 Jul 2026	Ibn Sina Hospital, Ompora, Budgam	400 patients documented	Provide accessible medical advice	400 free consultations	Event report
2	Medical check-ups and examinations	06–07 Jul 2026	Same venue	Patients attending camp	Assess health concerns	Examinations conducted; quantity [INFORMATION REQUIRED]	Event report / photos
3	Professional medical guidance	06–07 Jul 2026	Same venue	Patients attending camp	Support informed next steps	Guidance provided	Event report
4	Identification of further-care needs	06–07 Jul 2026	Same venue	Patients requiring further care	Identify treatment/surgical needs	Patients identified; count [INFORMATION REQUIRED]	Event report
5	Referral guidance	06–07 Jul 2026	Same venue	Patients requiring further care	Connect beneficiaries to appropriate care pathways	Referral guidance provided; referrals issued [INFORMATION REQUIRED]	Event report
6	Preventive-health messaging	06–07 Jul 2026	Same venue	Patients/community members	Promote regular check-ups and preventive care	Health-awareness messaging	Event report

9. Event / Activity Narrative

A. Context

The camp was organised as a direct community-health service intervention to improve access to medical consultation and examinations.

B. Objective

The intervention aimed to provide free consultation, support identification of health concerns, provide medical guidance, and direct patients needing further care to referral pathways.

C. Preparation

The available documentation confirms institutional collaboration but does not provide a detailed planning schedule, mobilisation plan, staffing roster or procurement record.

D. Implementation

Over 06–07 July 2026, patients received free consultations. The documented service pathway included check-ups, examinations, professional guidance, identification of further treatment or surgical needs, and referral guidance.

E. Participants

The documented beneficiary count is 400 patients receiving free consultation. Detailed demographic information is not available.

F. Key activities

Consultation, examination, medical guidance, identification of care needs, referral guidance and preventive-health messaging.

G. Community engagement

Community-oriented outreach is documented; the exact mobilisation mechanism and engagement count are not supplied.

H. Resources / materials

Medical professionals, healthcare facility, community outreach and institutional collaboration are documented as core inputs. Specific equipment, medicines, diagnostics and quantities are not documented.

I. Stakeholder involvement

Bhat Foundation Welfare Society is the implementing organisation and Ibn Sina Hospital is the documented healthcare partner.

J. Immediate observations

The available report describes improved access to free consultation, support for early identification, health awareness and referral support. These are reported programme outcomes, not independently measured long-term impact.

K. Outputs

The strongest quantified output is 400 free patient consultations over two programme days.

L. Follow-up actions

Referral completion, subsequent treatment and beneficiary follow-up should be tracked where records permit.

10. Outputs Achieved — Output Dashboard

Indicator	Planned	Achieved	Variance	Evidence
Patients receiving free consultation	Not specified	400	Not calculated	Event report
Programme days	Not specified	2	Not calculated	Event report
Healthcare partnership	Not specified	1 documented partner	Not calculated	Event report
Check-ups / examinations	Not specified	Conducted; quantity not recorded	Not calculated	Event report
Referral guidance	Not specified	Provided where further care was identified; count not recorded	Not calculated	Event report
Volunteers mobilised	Not documented	Not recorded in event documentation	N/A — planned figure not documented	Event documentation does not record a volunteer count
Medicines / diagnostics distributed	Not documented	Not recorded in event documentation	N/A — planned figure not documented	Event documentation does not quantify medicines or diagnostics distributed

Data integrity note: Missing figures are reported as "Not documented" rather than estimated or fabricated. Only quantities explicitly supported by the event documentation are presented as achieved.

11. Outcomes and Early Impact

Immediate outcomes

- Patients received access to free medical consultation.
- Medical examinations supported identification of health concerns.
- Patients requiring additional treatment or surgical intervention were identified and guided towards referral pathways.
- Preventive-health and regular-check-up messaging was delivered.

Short-term outcomes

The supplied documentation indicates improved access to professional advice and referral support. No post-camp survey, referral-completion dataset or follow-up clinical record was provided to quantify persistence of these outcomes.

Potential long-term impact

Potential longer-term effects may include earlier access to appropriate care and improved preventive-health awareness, but these should be treated as intended or plausible impacts rather than demonstrated results until follow-up evidence is available.

12. Impact Measurement Framework

Impact area	Indicator	Baseline	Target	Achieved	Evidence	Status
Healthcare access	Patients receiving free consultation	Baseline data not available	Not specified	400	Event report	Verified
Service delivery	Programme duration	Baseline data not applicable	Not specified	2 days	Event report	Verified
Early identification	Patients identified for further treatment/surgery	Baseline data not available	Not specified	Count not recorded	Event report	Partially Verified
Referral support	Referral guidance / referrals issued	Baseline data not available	Not specified	Guidance documented; count not recorded	Event report	Partially Verified
Health awareness	Preventive-care messaging	Baseline data not available	Not specified	Documented as delivered	Event report	Partially Verified

13. Beneficiary Stories / Human Impact

[INSERT VERIFIED BENEFICIARY STORY] — No individual beneficiary case story was supplied in the source material. The report therefore does not create a fictional case narrative.

14. Testimonials

No beneficiary testimonial was provided for this report.

Recommended evidence for future reporting: obtain informed consent, anonymised beneficiary identifiers where appropriate, the beneficiary's role/location, the service received, observed change, and a verified quotation.

15. Partnerships & Stakeholder Engagement

Stakeholder	Documented role / contribution	Evidence status
Bhat Foundation Welfare Society	Implementing / organising organisation	Verified from event report
Ibn Sina Hospital	Healthcare partner and venue; healthcare services delivered through the camp	Verified from event report
Government / other institutions	[INFORMATION REQUIRED]	Not provided
Community leaders / volunteers	[INFORMATION REQUIRED]	Not provided
Media partners	[INFORMATION REQUIRED]	Not provided

No endorsement should be inferred from the documented collaboration.

16. Volunteer Contribution

Volunteer indicator	Documented information
Number of volunteers	[INFORMATION REQUIRED]
Roles	[INFORMATION REQUIRED]
Hours contributed	[INFORMATION REQUIRED]
Community mobilisation	Community outreach documented; volunteer-specific contribution not quantified
Logistics / documentation	[INFORMATION REQUIRED]

17. Resource Utilisation

Resource category	Documented use	Quantity / value
Human resources	Medical professionals and healthcare staff	[INFORMATION REQUIRED]
Venue / facility	Ibn Sina Hospital	Documented
Community outreach	Used as an intervention input	Quantity [INFORMATION REQUIRED]
Medical equipment	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]
Medicines / diagnostics	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]
Transport	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]
Communication / printing	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]

18. Budget Utilisation

Budget Head	Approved Budget	Actual Expenditure	Variance	% Utilisation	Remarks
Programme / medical services	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	Financial records not supplied
Logistics	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	Financial records not supplied
Communication / documentation	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	Financial records not supplied
Total	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	

Financial utilisation details are maintained separately / not provided for this report.

19. Cost-Effectiveness

A verified cost-effectiveness calculation cannot be completed because documented expenditure was not supplied. Once expenditure and beneficiary figures are reconciled, the principal calculation can be

presented as: Total documented expenditure ÷ 400 documented direct service recipients = cost per documented direct beneficiary.

No cost per beneficiary, cost per session, cost per participant or resource-leverage ratio is reported in this version.

20. Monitoring & Documentation

The supplied evidence base includes a formal event impact report and photographic documentation. The report records the intervention, 400 free consultations, programme dates, venue, partnership, service pathway and forward monitoring recommendations.

Evidence No.	Evidence Type	Description	Date	Source / Reference	What it verifies
E-01	Event report	Bhat Foundation Free Multi-Specialty Medical Camp report	06–07 Jul 2026	Supplied event report	Programme scope, dates, venue, partner, 400 consultations, service pathway
E-02	Photographic documentation	Photographs showing medical camp activity / consultation setting	06 Jul 2026	Supplied photographs	Visual evidence of medical activity; date/location overlays require reconciliation
E-03	Photo metadata	GPS overlay showing Railway Station Road, Srinagar, 191111	06 Jul 2026	Supplied photographs	Photo timestamp/location metadata; conflicts with stated Ompora venue and requires verification

21. Photo Documentation

Photo	Caption / observation	Location	Date	Activity	What it demonstrates
Photo 01	Photograph supplied as part of camp documentation; exact visual content could not be reliably extracted from the standalone file.	[RECONCILE]	06 Jul 2026	Medical camp documentation	Use only after visual/original-file verification.
Photo 02	Photo file contains location text identifying Chak Kali Khan / near railway station, 191111.	Chak Kali Khan / railway-station area as displayed	[Not independently verified]	[INFORMATION REQUIRED]	Geographic metadata only; not sufficient to establish event venue.
Photo 03	Photo metadata records coordinates 34.0365976, 74.7367226 and 06 July 2026.	GPS overlay: Srinagar area	06 Jul 2026	[INFORMATION REQUIRED]	Timestamp and GPS metadata.

Photo	Caption / observation	Location	Date	Activity	What it demonstrates
Photo 04	Photo shows a medical-care setting and a GPS overlay reading Railway Station Road, Srinagar, 191111.	Railway Station Road, Srinagar, as displayed	06 Jul 2026	Medical consultation / examination setting	Visual evidence of healthcare activity; venue reconciliation required.
Photo 05	Photo metadata records Railway Station Road, Srinagar, 191111 and coordinates 34.0365976, 74.7367226.	Srinagar area as displayed	06 Jul 2026	[INFORMATION REQUIRED]	Photo metadata.

Photo-use recommendation
Before external donor publication, reconcile the GPS overlays with the official venue record. If the photographs were taken at the stated venue despite an incorrect GPS label, retain the original evidence and document the discrepancy rather than silently removing it.

Photographic Evidence

The following photographs were supplied as field evidence for the medical camp and document beneficiary attendance, clinical consultation, examination activity and the camp environment. They are included as supporting implementation evidence; they do not by themselves establish additional beneficiary counts or clinical outcomes.



Photo 1 — Medical consultation in progress at the camp.



Photo 2 — Beneficiaries awaiting and participating in medical services.

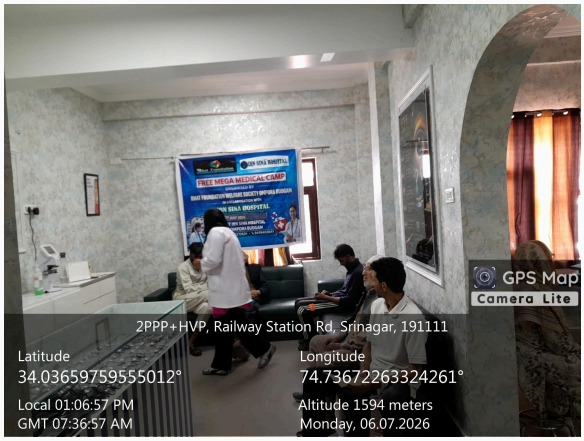


Photo 3 — Camp setting and beneficiary service area.

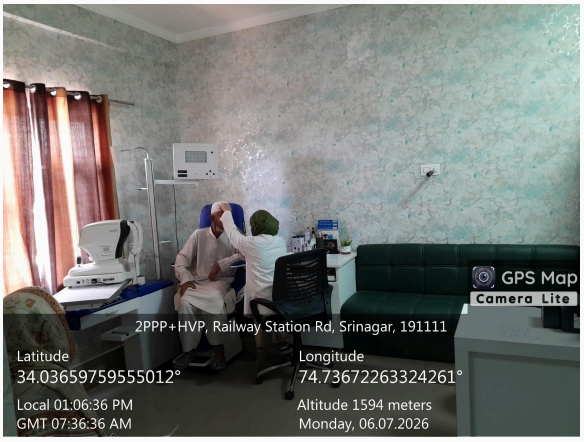


Photo 4 — Eye examination / clinical assessment being conducted.



Photo 5 — Clinical examination and beneficiary interaction.

Evidence note: The supplied photographs contain GPS overlays dated 06 July 2026 that display Railway Station Road, Srinagar, while the event report identifies Ibn Sina Hospital, Ompora, Budgam as the venue. This discrepancy is retained transparently in the report and should be reconciled against the original venue/attendance records before external donor publication.

22. Media & Digital Reach

Publication / platform	Date	Headline / post	Reach / engagement	Link	Relevance
Media / social media	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	[INFORMATION REQUIRED]	Not documented in supplied material

No media reach is claimed because no verified media or social-media analytics were supplied.

23. SDG Alignment

SDG	Relevant target	Project contribution	Evidence
SDG 3 — Good Health and Well-being	Target 3.8: access to quality essential healthcare services	The camp provided free medical consultation, examinations, health guidance and referral support.	Event report
SDG 10 — Reduced Inequalities	Relevant where access barriers to services are reduced	Free consultation can reduce an immediate cost barrier for participating patients; the extent of financial impact is not quantified.	Event report

The SDG mapping describes thematic alignment; it does not establish that the project independently achieved an SDG target.

24. ESG / CSR Alignment

Area	Documented alignment
Health / social development	Direct community-health service delivery
Community engagement	Community-oriented outreach and free access
Institutional partnership	Collaboration between welfare organisation and healthcare institution
Inclusion / access	Free consultation intended to reduce immediate access barriers
Governance / evidence	Programme dates, outputs and photographic documentation recorded

Legal CSR eligibility or statutory compliance is not assessed in this report.

25. Challenges Encountered

Challenge	Effect	Response	Lesson
Documentation gaps	Limited ability to quantify demographics, referrals, expenditure and follow-up	Mark missing data explicitly rather than estimate	Use a standard pre-/post-camp monitoring template
Photo-location discrepancy	Potential inconsistency between stated venue and GPS overlays	Reconcile original venue records before external publication	Validate metadata and evidence before donor submission
No quantified referral dataset	Longer-term care pathway cannot be measured	Track referrals issued and completed	Introduce a referral register with privacy safeguards

26. Lessons Learned

Operational lessons

- Record staffing, service stations, equipment and operating hours in the event log.

Community engagement lessons

- Record mobilisation channels and participant sources so reach can be understood beyond the headline attendance figure.

Monitoring lessons

- Capture baseline, planned targets, achieved values and referral follow-up in a single monitoring sheet.

Partnership lessons

- Document each partner's role and the specific resources or services contributed.

Future planning lessons

- Standardise evidence collection across camps to improve comparability and donor reporting.

27. Sustainability & Follow-up

The supplied report recommends continued community-based medical outreach, stronger beneficiary registration and follow-up, referral-completion tracking, anonymised beneficiary feedback and standardised pre-/post-camp indicators.

A verified long-term follow-up mechanism is not documented in the supplied material. Sustainability should therefore be presented as a forward plan rather than an established outcome.

28. Replication / Scalability

The intervention model is potentially replicable where a community organisation can secure a healthcare partner, qualified medical personnel, an appropriate facility, community mobilisation and a documented referral pathway. Replication should also include a defined monitoring system, privacy safeguards for beneficiary data, and a clear budget.

Requirement	Consideration for replication
Human resources	Qualified medical professionals and support staff; exact staffing model to be defined
Partnerships	Healthcare institution willing to provide venue and/or services
Community participation	Local mobilisation and beneficiary registration
Cost	Documented budget required before replication
Monitoring	Standard indicators for consultations, examinations, referrals and follow-up
Risks	Capacity constraints, documentation gaps and referral follow-through
Geographic expansion	Requires site-specific needs assessment and partner availability

29. Key Achievements

- Delivered a two-day free medical camp on 06–07 July 2026.
- **Documented 400 patients receiving free consultation.**
- Provided medical check-ups and examinations as part of the service pathway.
- Provided professional medical guidance and preventive-health messaging.

- Identified patients requiring further treatment or surgical intervention and provided referral guidance.
- Demonstrated collaboration between Bhat Foundation Welfare Society and Ibn Sina Hospital.

Each achievement above is limited to the evidence supplied; counts for examinations, referrals, staffing and financial resources remain unavailable.

30. Impact Scorecard

Category	Status	Basis
Reach	Verified	400 patients received free consultation
Participation	Partially Verified	400 service recipients documented; demographics unavailable
Activities	Verified	Consultation, examination, guidance and referral pathway documented
Outputs	Verified	400 free consultations; 2 programme days
Community Engagement	Partially Verified	Community outreach documented, but mobilisation metrics unavailable
Partnerships	Verified	Ibn Sina Hospital documented as healthcare partner
Documentation	Partially Verified	Event report and photographs supplied; photo-location discrepancy requires reconciliation
Follow-up	Data Not Available	Referral completion and post-camp outcomes not quantified

31. Before → Intervention → After

BEFORE — Problem / Need	INTERVENTION — What the NGO did	AFTER — Documented result
Potential access and cost barriers to medical consultation; quantified baseline unavailable.	Organised a two-day free medical camp with Ibn Sina Hospital; delivered consultation, examinations, health guidance and referral guidance.	400 patients received free consultation; further-care needs were identified where applicable; referral guidance was provided.

32. Theory of Change

Stage	Documented content
INPUTS	Medical professionals; healthcare facility; community outreach; institutional collaboration
ACTIVITIES	Medical consultations; patient examinations; health guidance; identification of concerns; referral guidance
OUTPUTS	400 patients received free consultation; two programme days
SHORT-TERM OUTCOMES	Improved access to consultation; support for early identification; health awareness; referral support
LONG-TERM IMPACT	Intended potential contribution to timely care and preventive-health behaviour; not demonstrated without follow-up evidence

33. Donor / CSR Value Proposition

The intervention provides documented evidence of direct community service delivery, local implementation, institutional healthcare collaboration and a measurable service output. The 400-consultation figure gives donors a clear, auditable starting point for understanding reach.

For future donor-supported iterations, the evidence base can be strengthened by tracking beneficiary demographics, staffing, service volumes, referrals issued and completed, expenditure, beneficiary feedback and follow-up outcomes. This would allow partners to assess both efficiency and sustained results rather than relying on attendance alone.

34. Future Recommendations

Immediate

Reconcile the stated venue with photographic GPS metadata.

- Attach attendance/registration records and the official event permission or venue confirmation, if available.
- Record exact number of medical professionals, volunteers, examinations and referrals.

3–6 months

- Introduce standard pre-/post-camp monitoring indicators.
- Track referral completion and subsequent treatment where records permit.
- Collect anonymised beneficiary feedback and case studies with consent.

6–12 months

- Develop a repeatable community medical-outreach model with standard budgeting and reporting.
- Build a longitudinal dataset across camps to compare reach, service volume, referral completion and beneficiary experience.

35. Conclusion

The Free Multi-Specialty Medical Camp conducted by Bhat Foundation Welfare Society in collaboration with Ibn Sina Hospital on 06–07 July 2026 was a direct community-health intervention focused on access to medical consultation. The supplied documentation records 400 patients receiving free consultation over two programme days.

The intervention also included medical check-ups and examinations, professional guidance, identification of patients requiring further care, referral guidance and preventive-health messaging. The evidence demonstrates immediate service delivery and a healthcare partnership, while longer-term impact remains to be measured through follow-up.

The report deliberately does not estimate missing financial, demographic, staffing, referral or outcome data. Strengthening those records in future camps will improve donor accountability, cost-effectiveness analysis and outcome measurement.

36. Annexures

Annexure	Status
Annexure I — Attendance / Registration Sheet	[TO BE ATTACHED]
Annexure II — Photographs	Available / supplied; venue metadata requires reconciliation
Annexure III — Certificates	[TO BE ATTACHED, IF APPLICABLE]
Annexure IV — Media Coverage	[TO BE ATTACHED, IF AVAILABLE]
Annexure V — Beneficiary Feedback	[TO BE ATTACHED]
Annexure VI — Bills / Invoices	[TO BE ATTACHED]
Annexure VII — Permission / Official Letters	[TO BE ATTACHED]
Annexure VIII — Social Media Documentation	[TO BE ATTACHED]
Annexure IX — Other Supporting Documents	[TO BE ATTACHED]

37. Final Donor Snapshot

Field	Documented value
PROJECT	Free Multi-Specialty Medical Camp
DATE	06–07 July 2026
LOCATION	Ibn Sina Hospital, Ompora, Budgam
DIRECT BENEFICIARIES	400 patients receiving free consultation
INDIRECT REACH	[INFORMATION REQUIRED]
VOLUNTEERS	[INFORMATION REQUIRED]
ACTIVITIES	Consultation, check-ups, examinations, guidance, further-care identification, referral guidance, preventive-health messaging
KEY OUTPUT	400 free patient consultations
KEY OUTCOME	Improved access to consultation; early-identification and referral support documented
PARTNERS	Ibn Sina Hospital
TOTAL DOCUMENTED COST	[INFORMATION REQUIRED]
SDGs	SDG 3; thematic alignment with SDG 10
EVIDENCE AVAILABLE	Event report and photographic documentation
FOLLOW-UP	Referral completion and beneficiary feedback tracking recommended

WHY THIS INTERVENTION MATTERS

The intervention matters because it documents a direct, community-level healthcare service in which 400 patients received free consultation over two days. The programme combined access to professional advice with examinations, health guidance and a pathway for patients who may require further treatment or surgical intervention. Its documented partnership between Bhat Foundation Welfare Society and Ibn Sina Hospital illustrates how community organisations and healthcare institutions can combine local outreach with clinical capacity. The evidence also highlights the importance of stronger follow-up measurement. Tracking referrals, beneficiary feedback, staffing, expenditure and demographic reach would allow future programmes to demonstrate not only how many people were served, but also what happened after the service and what resources were required to achieve the results.

FACT CHECK & DATA INTEGRITY REVIEW

Integrity check	Result
Every number is sourced	PASS — 400 patients and 2 programme days are taken from the supplied event report.
Every date is sourced	PASS — 06–07 July 2026 is documented in the supplied report; photo metadata separately records 06 July 2026.
Every beneficiary figure is sourced	PASS — 400 patients receiving free consultation.
Every partner is verified from provided material	PASS — Ibn Sina Hospital is identified as healthcare partner.
Every financial figure is sourced	PASS — no financial figures were invented; missing values are marked.
No testimonial was invented	PASS — report states none was provided.
No quotation was invented	PASS — no quotation used.
No impact claim exceeds available evidence	PASS — long-term impact is presented only as potential.
Outputs and outcomes are clearly separated	PASS.
Missing information is clearly marked	PASS — [INFORMATION REQUIRED] used where needed.
Evidence supporting major claims is identified	PASS — event report and photographic documentation are registered.
Calculations have been checked	PASS — no unsupported cost calculations were made.

Publication Hold Point
Before external donor/CSR circulation, reconcile the venue discrepancy visible in the photographic GPS overlays and attach the strongest available primary records (attendance, venue confirmation, referral register and financial statement). This is a documentation-control issue, not a reason to discard the documented programme output.

Source basis: supplied Bhat Foundation event impact report and supplied photographic documentation. This expanded report intentionally preserves unknowns rather than estimating them.